DringlichAI Demo Triage Guide

Mock priority guide for a software portfolio demonstration

THIS IS A MOCK DOCUMENT. It is fictional and was written only to demonstrate document ingestion, search and agent behaviour in the DringlichAI software project. It is not a medical protocol, has not been reviewed by any clinician, and must never be used to assess, prioritise or treat real patients.

1. About this guide

This guide is an original, simplified invention. It is not the Manchester Triage System or any other published triage protocol, and it does not reproduce any of their text, flowcharts or wording. The priority levels, waiting-time targets and discriminators below are made up so that a demo application has realistic-looking structured content to retrieve.

The demo application only suggests a priority level and the discriminators that triggered it. It never gives a diagnosis or treatment advice. A human nurse must confirm or override every suggestion.

2. Priority levels (demo values)

Level	Name	Demo target for first assessment
P1	Immediate	Straight away
P2	Very urgent	Within 15 minutes
P3	Urgent	Within 45 minutes
P4	Standard	Within 90 minutes
P5	Non-urgent	Within 180 minutes

Target times are fictional demo numbers, not clinical standards.

3. How to use this guide

Step 1. Choose the presentation that matches the main complaint (section 4).

Step 2. Work through that presentation's discriminators from the most urgent level to the least urgent.

Step 3. If a discriminator is confirmed, stop. The most urgent confirmed discriminator sets the level.

Step 4. If it is unknown whether a discriminator applies, ask the follow-up question shown beside it.

Step 5. Never state or imply a diagnosis. Report the level and the discriminators only.

Open design decision: what the application does when no discriminator matches is defined by the application, not by this guide.

4. Presentations and discriminators

Each row has a unique identifier, a plain-language discriminator, a demo priority level and a follow-up question the nurse can ask when the answer is not yet known.

Presentation CP: Chest pain

Use when the main complaint is pain, pressure or tightness in the chest.

ID	Discriminator	Level	Follow-up question for the nurse
CP-01	Chest pain with collapse or cannot be woken	P1	Has the patient collapsed or become hard to wake?
CP-02	Chest pain with severe breathing difficulty	P1	Can the patient speak a full sentence without stopping?
CP-03	Crushing or heavy pressure, spreading to arm, jaw or back, with sweating or pale skin	P2	Does the pain spread anywhere? Is the patient sweating or pale?
CP-04	Sudden severe pain that started within the last hour	P2	When exactly did it start, and how bad is it from 0 to 10?
CP-05	Pain that is worse when breathing in or moving, patient otherwise stable	P3	Does breathing or movement make it worse?
CP-06	Mild pain lasting several days, no other symptoms	P4	How long has it lasted, and are there any other symptoms?

Presentation SB: Shortness of breath

Use when the main complaint is difficulty breathing or breathlessness.

ID	Discriminator	Level	Follow-up question for the nurse
SB-01	Cannot speak, blue or grey lips, or exhausted from the effort of breathing	P1	Can the patient talk at all? What colour are the lips?
SB-02	Can only say a few words at a time	P2	How many words can the patient say before needing a breath?
SB-03	Wheezing, known asthma, speaks in full sentences	P3	Is there a known breathing condition, and has the usual inhaler helped?
SB-04	Mild breathlessness on exertion, present for several days	P4	Does it happen only on effort, and for how many days?
SB-05	Cold symptoms with mild cough, breathing normal	P5	Is the breathing comfortable at rest?

Presentation HA: Headache

Use when the main complaint is a headache.

ID	Discriminator	Level	Follow-up question for the nurse
HA-01	Headache with reduced alertness or fits	P1	Is the patient drowsy, hard to wake, or has there been a fit?
HA-02	Sudden headache described as the worst ever	P2	Did it come on suddenly, and is it unlike any previous headache?
HA-03	Headache with fever and stiff neck, or new weakness or confusion	P2	Is there a fever, a stiff neck, new weakness or confusion?
HA-04	Moderate headache with vomiting, known migraine history	P3	Is this like previous migraines? Any vomiting?
HA-05	Mild headache, no other symptoms	P5	Are there any other symptoms at all?

Presentation AB: Abdominal pain

Use when the main complaint is pain in the belly.

ID	Discriminator	Level	Follow-up question for the nurse
AB-01	Abdominal pain with collapse or cannot be woken	P1	Has the patient collapsed or become hard to wake?
AB-02	Severe pain, or vomiting blood	P2	How bad is the pain from 0 to 10? Has there been any blood in vomit?
AB-03	Moderate pain with repeated vomiting	P3	How many times has the patient vomited, and can they keep fluids down?
AB-04	Pain with diarrhoea, patient able to drink	P4	Is the patient able to drink normally?
AB-05	Mild pain for several days, eating normally	P5	Is the patient eating and drinking normally?

Presentation WI: Minor injury and wounds

Use when the main complaint is a cut, graze, sprain or other minor injury.

ID	Discriminator	Level	Follow-up question for the nurse
WI-01	Heavy bleeding that does not stop with firm pressure	P1	Has the bleeding stopped after several minutes of firm pressure?
WI-02	Deep wound, visible bone, or a limb that looks deformed	P2	Can you see bone, or does the limb look out of shape?
WI-03	Cut that is likely to need closing	P3	Is the cut deep or gaping, and is it still bleeding?
WI-04	Sprain or twist, patient can put weight on the limb	P4	Can the patient stand and put weight on it?
WI-05	Graze or small bruise	P5	Is there any other injury or symptom?

5. Reminder

This guide is a mock created for software demonstration. Real triage must follow a licensed, clinically validated protocol applied by trained staff. If you are unwell or worried about someone's health, contact a doctor or emergency services.